

# BIOS 794: Unsupervised Learning: Clustering

Alexander McLain

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# Clustering

- ▶ The goal is to identify *natural groupings* in the data without relying on an outcome variable.
- ▶ Clustering is an unsupervised learning technique that is one of the most natural exploratory data analysis (EDA) methods.
- ▶ Examples in public health:
  - ▶ Grouping patients by symptom patterns or biomarker profiles.
  - ▶ Identifying communities with similar socioeconomic or health characteristics.
  - ▶ Categorizing hospitals based on performance indicators.

# Clustering

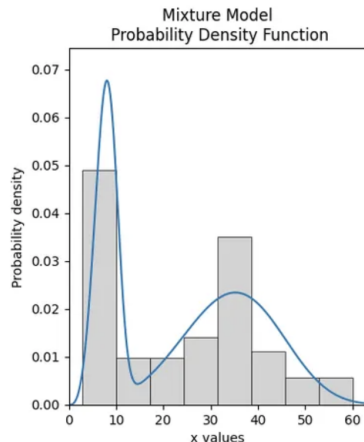
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- ▶ Examples in public health:
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  - ▶ Identifying communities with similar socioeconomic or health characteristics.
  - ▶ Categorizing hospitals based on performance indicators.
- ▶ **Differences from classification**
  - ▶ we don't know the number of classes
  - ▶ we don't know anybody's class (no ground truth)
  - ▶ the goal is more exploratory and less predictive
  - ▶ we can cluster observations (i.e., the  $Y_i$ 's), the predictor variables (i.e., the  $X_i$ 's), or both.

## Definition

- ▶ Defining what a cluster is can be difficult and seems subjective.
- ▶ Statistically, we think of clusters as a mixture distribution:

$$p(\mathbf{X}|\Phi) = \sum_{k=1}^K I(\mathbf{X} \in \mathcal{C}_k) p_k(\mathbf{X}|\phi_k).$$

- ▶ For example, the  $f_k(\mathbf{X}|\phi_k)$  could be, for example, different multivariate normal distributions.



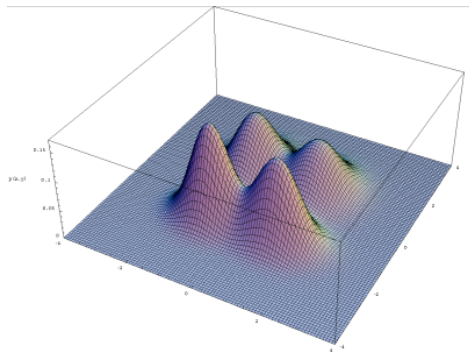


Figure: Multivariate Mixture of Gaussian distributions.

## Dissimilarity

- ▶ Many clustering algorithms rely on the dissimilarity (or distance) metric used.
- ▶ Let's think of  $\mathbf{x}_i$  and  $\mathbf{x}_j$  as the biomarker profiles (e.g., immune response measures) between patients  $i$  and  $j$ .
- ▶ The dissimilarity between  $\mathbf{x}_i$  and  $\mathbf{x}_j$  has the following properties
  1.  $d(\mathbf{x}_i, \mathbf{x}_j) \geq 0$
  2.  $d(\mathbf{x}_i, \mathbf{x}_j) = 0 \Leftrightarrow \mathbf{x}_i = \mathbf{x}_j$
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- ▶ **Examples**
  - ▶ Euclidean:  $d(\mathbf{x}_i, \mathbf{x}_j) = \sqrt{\sum_{k=1}^p (x_{ik} - x_{jk})^2}$
  - ▶ Manhattan:  $d(\mathbf{x}_i, \mathbf{x}_j) = \sum_{k=1}^p |x_{ik} - x_{jk}|$
  - ▶ 1-correlation:  $d(\mathbf{x}_i, \mathbf{x}_j) = 1 - \rho_{ij}$

## K-means: Concept

- ▶ One of the most widely used clustering algorithms.
- ▶ Objective: divide data into  $K$  groups (clusters) that minimize within-cluster variability (i.e., the within-cluster distances).
- ▶ Each observation is assigned to the cluster with the *nearest* mean (centroid).

### Example:

- ▶ Suppose we have county-level data on obesity rate, smoking rate, and median income.
- ▶ K-means can group counties into clusters that share similar health risk profiles.



## K-means: Objective Function

- ▶ K-means minimizes the total within-cluster sum of squares:

$$\min_{C_1, \dots, C_K} \sum_{k=1}^K \sum_{x_i \in C_k} \|x_i - \mu_k\|^2$$

where  $\mu_k$  is the mean of cluster  $C_k$ .

- ▶ Algorithm steps:
  1. Choose  $K$  (number of clusters).
  2. Randomly assign points to clusters.
  3. Compute cluster means.
  4. Reassign points to the nearest mean.
  5. Repeat until assignments stop changing.

## Choosing $K$

- ▶ For all of the clustering algorithms, we need to specify  $K$ , the number of clusters in the given dataset.
- ▶ Common methods:
  - ▶ **Elbow method:** plot total within-cluster sum of squares vs  $K$ .
  - ▶ **Gap Statistic:** measures when the improvement in fit is not better than what is expected by chance.
- ▶ In public health, interpretability also matters—choose  $K$  that makes sense in context (e.g., 3-5 community profiles).

## Elbow method

- ▶ Let  $W_k$  be equal to the within-cluster distances for a given algorithm when the number of clusters is  $k$ .
- ▶ Let  $k^*$  be the true number of clusters.
- ▶ We'd expect that

$$\{W_k - W_{k+1} | k < k^*\} \gg \{W_k - W_{k+1} | k \geq k^*\}$$

.

- ▶ The difference in consecutive  $W_k$  values will be larger when  $k < k^*$ .
- ▶ An estimate of  $K$  is then obtained by identifying a “kink” in the plot of  $W_K$  as a function of  $K$ .

## Gap Statistic

- ▶ A method to determine if  $W_k - W_{k+1}$  is “large” or “small” is the Gap Statistic.
- ▶ This is a Monte Carlo method, multiple datasets with no clusters are generated with a similar spread to the variables in the original data.
- ▶ **Random Gap Values:** the  $W_k - W_{k+1}$  values from these *random* datasets
- ▶ Use the Random Gap Values to judge if the observed values are larger than what would be expected by chance.

## Gaussian Mixture Models (GMMs)

- ▶ K-means assumes spherical clusters of similar size.
- ▶ Gaussian Mixture Models (GMMs) assume each cluster follows a multivariate normal distribution.
- ▶ Each observation has a **probability of belonging** to each cluster.

### Example:

- ▶ In a nutrition dataset, GMMs can identify overlapping groups such as “mostly healthy eaters” vs “moderate” vs “unhealthy.”

## Expectation-Maximization (EM) Algorithm

- ▶ Used to estimate GMM parameters.
- ▶ Alternates between:
  - ▶ **E-step:** Estimate cluster membership probabilities.
  - ▶ **M-step:** Update cluster means and covariances based on probabilities.
- ▶ Stops when parameter changes are very small.
- ▶ Output includes both the most likely cluster assignment and the uncertainty of membership.

## Partitioning Around Medoids (PAM)

- ▶ Similar to K-means but uses **medoids** (actual observations) instead of means.
- ▶ More robust to outliers because medoids minimize total distance within clusters.
- ▶ Works with any distance measure (not just Euclidean).

### Example:

- ▶ PAM could be used to cluster hospitals using a mix of numeric and categorical performance indicators.

# Hierarchical Clustering

- ▶ Hierarchical Clustering is a procedure that results in different degrees of clustering.
- ▶ There are two ways to do this:
  1. **Agglomerative:** Bottom-up: each point starts as its own cluster and ends up in one cluster.
  2. **Divisive:** Top-down, everyone starts as one cluster and ends up in their own cluster.
- ▶ These have similarities to forward-backward selection methods.
- ▶ For either method, observations are joined (or separated) by how dissimilar they are.



## Hierarchical Clustering

Algorithm for agglomerative clustering:

1. Everyone starts in their own cluster.
2. Find the smallest  $d_{ij}$ , say  $d_{IJ}$ . Put  $I$  and  $J$  in the same cluster.

Now we have to recompute the dissimilarities for the new group  $IJ$ .

3. Compute dissimilarities  $d_{IJ,k}$  for all  $k \neq I$  or  $J$ . Three options for doing this:
  - ▶ **single**:  $d_{IJ,k} = \min(d_{Ik}, d_{Jk})$
  - ▶ **complete**:  $d_{IJ,k} = \max(d_{Ik}, d_{Jk})$
  - ▶ **average**:  $d_{IJ,k} = \text{avg}(d_{Ik}, d_{Jk})$
4. Repeat steps 2–3  $n - 1$  times.

## Hierarchical Clustering

- ▶ Single is more likely to cluster an observation with a group of observations.
- ▶ Complete is more likely to create new clusters.
- ▶ The algorithm for divisive clustering begins with the entire data set as a single cluster, and recursively divides one of the existing clusters into two daughter clusters at each iteration in a top-down fashion.
- ▶ We won't discuss how this works, and divisive clustering is less common than agglomerative.

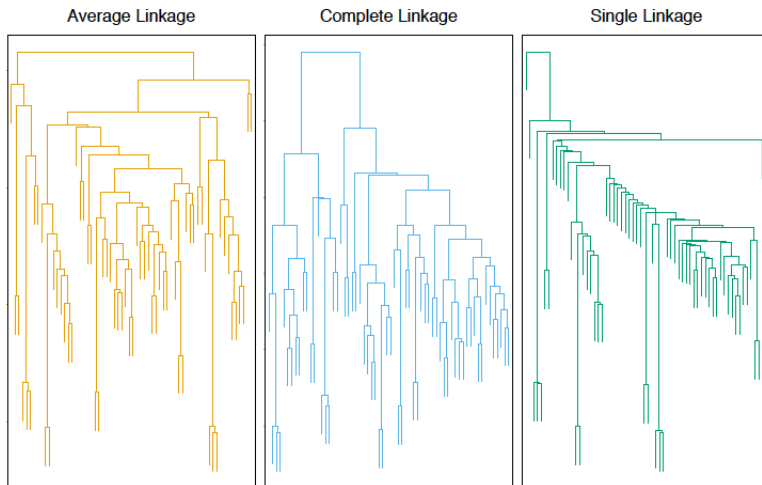


Figure: From page 524 in the online version of ESL.

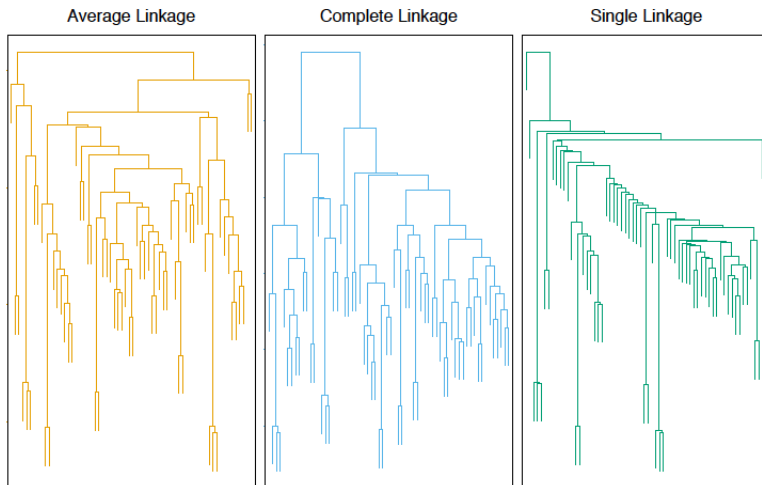


Figure: From page 524 in the online version of ESL.

## Using Clustering in Downstream Models

- ▶ In practice, clustering results are often used as predictors or group indicators in later analyses.
- ▶ **Examples:**
  - ▶ Use cluster membership as a covariate in a regression model for predicting disease risk.
  - ▶ Compare outcomes (e.g., mortality, quality of life) across clusters identified from biomarker or behavior data.
  - ▶ Use clusters to define subgroups for targeted interventions or policy evaluation.
- ▶ This approach allows us to connect unsupervised patterns with meaningful outcomes.
- ▶ **However**, it changes the scientific question: we are no longer just describing structure, but testing hypotheses *about the clusters themselves*.

## Pitfalls and Recommendations

- ▶ **Main Concern:** The same data are used to *create* and *test* clusters.
- ▶ This can lead to:
  - ▶ Overstated associations (data snooping bias)
  - ▶ Invalid p-values or confidence intervals
  - ▶ Misleading conclusions about “differences” between clusters
- ▶ **Good Practices:**
  1. If possible, use an independent dataset to test relationships between clusters and outcomes.
  2. Use *cross-validation* or bootstrapping to assess the stability of the clusters.
  3. Treat clustering results as *exploratory*—use them to generate, not confirm, hypotheses.
  4. Report how sensitive your downstream results are to the chosen number of clusters and method.

## When to Use Each Method

- ▶ **K-means:** fast, works well for continuous data with spherical clusters.
- ▶ **GMM (model-based):** captures overlapping and elliptical clusters.
- ▶ **PAM:** robust to outliers, works with mixed data types.
- ▶ **Hierarchical:** good for exploring structure or visualizing relationships.

**Key takeaway:** Always pair clustering with interpretation and domain expertise—machine learning finds structure, but you decide if it makes sense.

## Summary

- ▶ Clustering is essential for exploratory analysis in public health and many other fields.
- ▶ Always pre-process data: scale variables, handle outliers, and ensure numeric format.
- ▶ Use visualization to interpret results (e.g., scatter plots, silhouette plots, dendrograms).
- ▶ Combine statistical performance and interpretability when deciding the number of clusters.
- ▶